NCCP 00563 · VERSION 4

Your treatment, explained

NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation. · NCCP 00563 v4 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Encorafenib and binimetinib for advanced BRAF-mutated melanoma

Reference calendar for daily medicine and reviews

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
The overall treatment plan
Your 28-day timeline
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UNDERSTAND YOUR TREATMENT

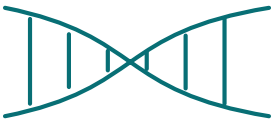
Medicines in this regimen

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

ORAL BRAF INHIBITOR

Encorafenib

Braftovi



Blocks altered BRAF signalling

Encorafenib blocks the BRAF V600 growth signal in eligible melanoma cells. Binimetinib acts further down the same pathway. They have different dose schedules and different safety decisions, so the prescription for one must not be assumed to apply to the other.

BRAF V600 confirmed → BRAF signal blocked → Partner MEK medicine

What to watch for

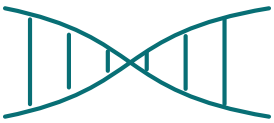
A change to binimetinib can change the encorafenib plan. Have interactions checked and avoid grapefruit.

Sources: nccp · nccn-patient

ORAL MEK INHIBITOR

Binimetinib

Mektovi



Blocks downstream MEK signalling

Binimetinib inhibits MEK in the cell-growth pathway. It can also affect normal muscle, heart and retinal tissues. CK blood tests, heart checks and prompt reporting of eye or muscle changes help the team identify problems before they become more serious.

MEK inhibited → Cancer-growth signal reduced → Muscle / heart / eye checks

What to watch for

Report vision change or muscle pain/weakness promptly; keep CK, kidney and heart checks even when you feel well.

Sources: nccp · nccn-patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Energy / fever

Both medicines

Fatigue and headache can occur. Fever, marked weakness, dizziness or new neurological symptoms needs advice; do not assume it is a harmless drug effect.

3 Heart / circulation

Both medicines

Keep ECG and heart-pumping checks. Report palpitations, fainting, swollen ankles or new breathlessness; severe chest pain is an emergency.

5 Liver

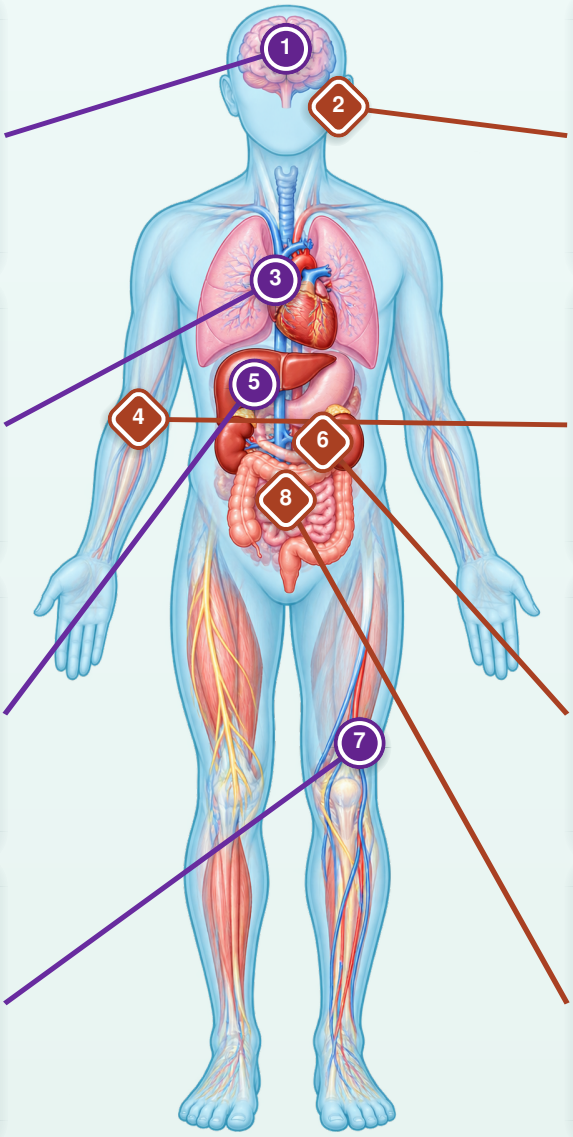
Both medicines

Liver changes may have no early symptoms, so attend monthly tests. Report yellow skin or eyes, dark urine or upper-abdominal pain.

7 Muscles / strength

Binimetinib

CK can rise before or with muscle injury. Report muscle pain, weakness or dark urine, and keep the planned blood tests.



2 Eyes / vision

Both medicines

Blurred vision, spots, altered colour vision or eye pain needs prompt assessment. Retinal problems may occur with binimetinib.

4 Skin / new lesions

Both medicines

Report new growths or changes in existing spots and attend skin checks. Severe rash, blistering or peeling needs urgent help.

6 Kidneys / fluids

Binimetinib / combination

Kidney tests accompany CK monitoring. Reduced urine, dehydration or dark urine with muscle symptoms needs urgent advice.

8 Stomach / bowels

Both medicines

Nausea and diarrhoea may occur. Persistent vomiting, inability to drink, severe abdominal pain, black stools or bleeding needs assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

The overall treatment plan

DAILY MEDICINES

Continuous oral treatment

No preset cycle total | **Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly.**

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00563 v4: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Encorafenib and binimetinib for advanced BRAF-mutated melanoma: treatment and checks

NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation.

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00563	Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, ECG, a heart-pumping assessment by echo or MUGA, and skin examination.	Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.
Cancer, skin and daily function NCCP 00563	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions NCCP 00563	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

What each check is for

1

Continuous oral treatment

Daily medicines

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00563

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily oral treatment	Days 1–28	Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly.
Safety reviews as scheduled	Days 1–28	Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.

Treatment or check	Days in this window	What happens
Symptoms and support	Days 1–28	Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

Timing note 1: Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Timing note 2: For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

Timing note 3: Temporary holds may need coordinated changes to both medicines; no patient-led dose adjustment.

Timing note 4: The regimen is advanced melanoma; it is not an adjuvant treatment template.

Timing note 5: No trial outcome percentage is assigned without confirming the exact population, comparison and follow-up.

Timing note 6: Tumour lysis syndrome is a source-listed risk, especially with high tumour burden, kidney problems or dehydration. The team decides whether extra hydration and blood checks are needed.

> **Why these bands stay broad:** This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00563 v4: current regimen source ↗

> **There are no guaranteed "safe days".** The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–28: Every day in the reference calendar

Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual method is to swallow the prescribed capsules/tablets whole with water, with or without food. If swallowing is difficult, ask the pharmacist for the source-permitted preparation: Braftovi capsules may be opened into about 20 mL of apple puree for immediate use, and Mektovi 15 mg tablets may be dispersed in about 10 mL of water, orange juice or apple juice, taken immediately, followed by a further 10 mL rinse. Do not assume this applies to other tablet strengths or improvise a preparation. Keep doses at consistent times. Avoid grapefruit and grapefruit juice, and have all medicines and supplements checked.

STEP 2

Missed, vomited or held treatment

For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

STEP 3

Appointments and supply

Arrange dates, medicine supply, transport and any needed interpreter or carer support. Ask whether bloods are taken in the oncology unit or elsewhere and how the result reaches the prescriber.

STEP 4

Paper copies and contact information

Keep this complete handbook, the three-page summary and the wallet card together. The QR code is optional. Ask the team to add your actual contacts and print a revised plan when treatment changes.

Sources: NCCP 00563 v4: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 Pregnancy and fertility precautions

Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team’s breastfeeding instructions. Report a possible pregnancy promptly.

4 Eating, activity and practical help

Report falling intake, weight loss, pain or fatigue early. The team can tailor nutrition, activity, wound, psychological and practical support to your needs.

5 Hydration and tumour lysis assessment

The team checks whether tumour burden, kidney problems or dehydration call for additional precautions and blood tests. Keep the individual fluid plan; do not force fluids if you have a restriction. Report reduced urine, persistent vomiting or marked weakness promptly.

Sources: nccp · nccn-patient

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Keep the pathology summary, original skin site, stage and details of any lymph-node involvement together. Breslow thickness and ulceration describe features of the original melanoma; they are different from the stage. If the diagnosis arose in the eye or a mucosal surface, ask whether this cutaneous-melanoma information applies.

Sources: nccn-patient

BRAF and other tumour tests

A BRAF V600 result helps decide whether the BRAF/MEK targeted medicines in this guide are suitable. This is usually a test of the tumour, not proof of an inherited family condition. Keep the actual result and test date; ask about genetic counselling only when your personal or family history makes that relevant.

Sources: nccn-patient

Surgery, radiotherapy and medicine have different roles

Record what surgery removed and whether the present medicine is postoperative treatment to lower recurrence risk or treatment of cancer that remains or has spread. Radiotherapy, stereotactic treatment or another operation may have a separate role; their timing and side effects need their own plan.

Sources: nccn-patient

Stage and response are different

Stage describes the extent of melanoma at diagnosis or a later reassessment. A response assessment asks how known cancer has changed with treatment. In the adjuvant setting there may be no visible tumour to measure: surveillance looks for recurrence while safety checks look for medicine effects.

Sources: nccn-patient

A new skin change still needs review

Continue to report a new or changing mole, a non-healing lesion, a new lump or an unexplained persistent symptom. Do not assume every new lesion is melanoma, and do not use a photograph or a good response at one site to rule out a problem elsewhere.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and the overall plan

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

The schedule in this setting

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Sources: nccp

How the treatment works

Encorafenib blocks the BRAF V600 growth signal in eligible melanoma cells. Binimetinib acts further down the same pathway. They have different dose schedules and different safety decisions, so the prescription for one must not be assumed to apply to the other. Binimetinib inhibits MEK in the cell-growth pathway. It can also affect normal muscle, heart and retinal tissues. CK blood tests, heart checks and prompt reporting of eye or muscle changes help the team identify problems before they become more serious.

Sources: nccp

The role of earlier or later treatment

Record surgery, radiotherapy and other anticancer medicines separately, including dates and the specialist responsible. This schedule does not automatically include or authorise another treatment. Ask whether any overlap changes tests, symptoms to report or the timing of appointments.

Sources: nccn-patient

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Sources: nccp

TREATMENT REFERENCE · 03

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

The usual method is to swallow the prescribed capsules/tablets whole with water, with or without food. If swallowing is difficult, ask the pharmacist for the source-permitted preparation: Braftovi capsules may be opened into about 20 mL of apple puree for immediate use, and Mektovi 15 mg tablets may be dispersed in about 10 mL of water, orange juice or apple juice, taken immediately, followed by a further 10 mL rinse. Do not assume this applies to other tablet strengths or improvise a preparation. Keep doses at consistent times. Avoid grapefruit and grapefruit juice, and have all medicines and supplements checked.

Sources: nccp

Missed, vomited or held treatment

For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

Sources: nccp

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Sources: nccp

Practical preparation

Arrange medicine supply, transport, interpreter or carer support before it is needed. Bring the alert card, symptom record and questions. Ask which appointments require someone to accompany you and how to obtain urgent advice outside normal hours.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, ECG, a heart-pumping assessment by echo or MUGA, and skin examination.

Sources: nccp

During treatment

Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan.

Sources: nccp

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Sources: nccp

After a pause or finish

Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.

Sources: nccp

Getting the result

For each test, record its purpose, due date, result and action. Agree how results reach you and who checks a result from another hospital. If an appointment or result has not arrived, contact the named service rather than assuming no news means no action is needed.

Sources: nccn-patient

TREATMENT REFERENCE · 05

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Sources: nccp

When to call

Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

Sources: nccp

Eating, drinking and activity

Tell the team early about nausea, taste change, a sore mouth, diarrhoea or falling weight. Ask for an individual fluid and food plan if heart, kidney or other conditions limit intake. Gentle activity and planned rest can help maintain function; adjust to symptoms and ask for support when fatigue limits daily life.

Sources: nccn-patient

A held dose and a new phase

For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

Sources: nccp

TREATMENT REFERENCE · 06

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Use shade, protective clothing, a hat and broad-spectrum sunscreen of at least SPF 30, and avoid tanning beds. Reapply sunscreen according to its instructions. Protection matters even when it is cloudy; choose a practical approach that lets you remain active safely.

Sources: nccn-patient

Know the pattern of your skin

Ask how to examine your skin, scars and relevant lymph-node areas and how often your own team recommends doing so. A mirror or a trusted helper can make hard-to-see areas easier to check. Report a new or changing lesion, a non-healing sore or a new lump rather than waiting for a routine visit.

Sources: nccn-patient

Photographs are optional

If useful, keep a dated photograph with consistent lighting and a size reference, following your team's advice. Photographs are optional and do not diagnose a cancer or replace examination. Keep them privately; this printable passport does not require you to upload images or other personal health information.

Sources: nccn-patient

Wounds, nails and sensitive skin

Use the dressings and skin-care plan recommended for your lesion and treatment. Ask before applying new creams or remedies to an open or treated area. Worsening pain, discharge, redness, bleeding or loss of function needs advice; routine skin-care instructions are not a substitute for assessment.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

Sources: nccp

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Sources: nccp

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Sources: nccp

TREATMENT REFERENCE · 08

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what evidence applies to your diagnosis, stage, biomarker and treatment setting, what outcome the team hopes to improve, and what may happen without this treatment. A trial average cannot predict an individual result; a benefit measured in one setting should not be transferred to another.

Sources: nccn-patient

Alternatives and review points

Discuss other suitable treatments, clinical trials if relevant, local treatment options and symptom-focused care. Ask when the decision will be reviewed and what would prompt a change. Supportive and palliative care can help with symptoms and quality of life alongside anticancer treatment.

Sources: nccn-patient

Help with daily life

Tell the team about work, transport, caring responsibilities, cost concerns, body image, anxiety or sleep. Ask which nurse, social worker, dietitian, rehabilitation professional or support organisation can help. Use resources relevant to your country and service; a US guide may describe different access or funding.

Sources: nccn-patient

Shared understanding

Ask someone on the team to check that you can explain the medicine schedule, next tests and urgent-contact route in your own words. An interpreter, large-print copy or a trusted supporter can help; none requires you to use a smartphone.

Sources: nccn-patient

TREATMENT REFERENCE · 09

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Sources: nccn-patient

Continuing medicine precautions

Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.

Sources: nccp

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

Sources: nccp

The complete paper version

This handbook can be used without a phone: keep the schedule, symptom advice, medicine list, tests and contacts together. The QR code is an optional route to the matching regimen information. Ask the team to print updates when your plan changes and keep the latest dated copy.

Sources: nccn-patient

What the evidence means for your treatment

The evidence discussion must match the diagnosis, biomarker and treatment setting shown in this guide.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

Sources: nccp · nccn-patient

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of my treatment?

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

Is there a fixed number of cycles?

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

What happens if treatment is held?

For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

When are my next tests?

Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00563 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00563 v4: current regimen source
- NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6744/563_Encorafenib-and-Binimetinib-Therapy.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf